CONTENT BANK · READY TO SCRIPT

10 Pillars · 50 Subtopics · 150 Angles

Dr. Azka's personal-brand content, built from your research. Every angle includes the spoken hook, on-screen text, 3 talking points, CTA, format and funnel stage, so each one can go straight into a script.

Sources: your positioning, ICP, business model, funnel/offer and tonality documents (Files 1–7) · 210 keyword-research reels across 21 keyword clusters · 76 competitor reels (Amra Farooq, Sam R Patel, Dr. Nickc, Maha Chalawi, Mrinal Tiwari, Akash Gupta, Shivam Bansal, Joshua Navaro, Dr Sakshi, Jakeyla Reed, Eshaan).
Audience: clinic-owner dermatologists, cosmetic dentists and aesthetic physicians · UAE + USA · Prepared 31 October 2026

How to read each angle

1.1.1 The spoken hook: the first words out of your mouth (0–1.5 s)

On-screen text: the big written hook in frame 1

- Talking point 1: the stakes or the first idea
- Talking point 2: the payload (steps / demo / example)
- Talking point 3: the proof or payoff line

CTA: the one action you ask for

Format · STAGE

Funnel stages: TOFU discovery: reach and follows · MOFU trust: clinic owners start believing you · BOFU conversion: audits, calls, clients.

'Built on' under each subtopic shows which keyword cluster or competitor reels it comes from, and the gap it fills.

Pillars and content mix

#	Pillar	Share of posts
1	What Dermatologists, Cosmetic Dentists & Aesthetic Doctors Should Actually Post	15%
2	Hooks, Scripting & Value-Per-Second (The Craft)	10%
3	Views → Patients (Content as a Patient-Acquisition Channel)	15%
4	Patient Psychology: How Patients Choose a Dermatologist or Cosmetic Dentist	10%
5	Evidence-Based & Compliant Medical Marketing (UAE + USA)	5%
6	The Patient Leak: Inquiry → Booking (Speed, DMs, Front Desk)	10%
7	AI Systems for Clinics (Receptionist, Booking, No-Shows)	15%
8	Retention, Reactivation & Reviews (Money Already in the Database)	5%
9	Clinic Growth Economics, Teardowns & Proof	5%
10	Dr. Azka: A Doctor Building for Doctors (Human Layer)	10%

Scripting rule for every angle (value per second)

Hook in 1.5 s, with visual + text + voice saying the same thing → stakes → payload, one idea per line → proof → one CTA. Every sentence must be a claim, a number, an example or a step. If you can delete a sentence and nothing is lost, delete it.

Compliance

Angles that mention regulators or rules (DHA, DoH, MOHAP, UAE advertiser permits, FTC, HIPAA, state boards, US consent rules) are educational. Check each one against the regulator's current guidance before filming, and add 'not legal advice' to the caption. Use real patient images or stories only with written consent.

PILLAR 1 OF 10 · 15% OF POSTS

What Dermatologists, Cosmetic Dentists & Aesthetic Doctors Should Actually Post

Your biggest authority pillar. It proves you know their specialty and their patients.

Research basis: Keyword clusters: 'Dermatologist content ideas', 'Dentists Instagram marketing', 'Medspa/aesthetic clinic social media marketing', 'medical content marketing'. Competitors: Amra Farooq, Akash Gupta ('5 pillars'), 'Content ideas for a med spa', 'Content ideas for dentists'. What they all miss: no finished, specialty-specific examples, generic 'doctors', ideas locked behind 'comment CONTENT'.

1.1 Specialty content blueprints

Built on: 'If I had to take a Dentist... 30 days' and 'Content ideas for dentists/med spa' reels. Their gap: dense lists, no sample script or calendar. You show the finished example.

1.1.1 If I ran a Dubai dermatology clinic's Instagram, these are the only 5 videos I'd post this month.

On-screen: “5 videos I'd post as a Dubai derm clinic”

- 1) 'What I'd do for my own pigmentation' (trust). 2) A myth about a viral skincare trend (reach).
- 3) A consultation walkthrough (removes booking fear). 4) A 'who is NOT a candidate' video (credibility).
- 5) A recovery-day reality video for a high-ticket laser (desire + honesty). Show one sample hook for each on screen.

CTA: Comment DERM and I'll send the 5 hooks written out.

Talking head + list · TOFU

1.1.2 Cosmetic dentists: 4 videos that sell veneers without ever showing a price.

On-screen: “Sell veneers. No price.”

- Video 1: 'What natural-looking veneers actually look like up close' (kills the 'fake teeth' fear).
- Video 2: the 'consultation in 40 seconds' walkthrough. Video 3: 'who shouldn't get veneers'.
- Video 4: a patient-journey day-by-day (with consent). Price questions get answered in the DM, not the post.

CTA: Save this before you plan next month's posts.

Green-screen + examples · MOFU

1.1.3 Aesthetic doctors: the 'treatment explained in 30 seconds' series that books consultations.

On-screen: “1 treatment. 30 seconds. 1 booking.”

- Template: what it is → who it's for → what it feels like → downtime → one honest limitation.
- Show it applied to Botox, Morpheus8 and skin boosters, with one line each on screen.
- Why a series beats one-offs: patients binge a series and come back to your profile.

CTA: Comment SERIES for the template.

Template breakdown · MOFU

1.2 Treatment-led content (selling high-ticket treatments)

Built on: dermatology product/routine reels (Celimax, dry skin routine) that lack safety context and candidacy. Your gap: candidacy + honesty = trust.

1.2.1 Before a patient books laser, they ask 3 questions. Your video should answer all three.

On-screen: “The 3 questions before every laser booking”

- Q1: Will it work for MY skin type? (candidacy, especially relevant for darker skin tones in the UAE).
- Q2: How much does it hurt, and how long until I look normal? (pain + downtime).
- Q3: How many sessions until I see results? Show the script structure for answering all three in 30 seconds.

CTA: Follow for one treatment script every week.

Script on screen · MOFU

1.2.2 The 'who is NOT a candidate' video: why telling patients no makes the right ones book.

On-screen: “Who should NOT get this treatment”

- Patients trust a doctor who turns people away; it signals integrity.
- Example structure: 'If you have X, Y or Z, this isn't for you. Here's what I'd suggest instead.'
- It pre-qualifies inquiries, so the front desk gets better leads.

CTA: Send this to the doctor who handles your content. Talking head · TOFU

1.2.3 Implants vs. veneers vs. aligners: how to turn patient confusion into 10 videos.

On-screen: “1 confusing question = 10 videos”

- Patient confusion is a content goldmine. Every comparison is a video.
- List the 10 on screen: cost range, longevity, pain, timeline, who it suits, myths, and so on.
- End with the cluster logic: 10 videos on one decision = a patient who's ready at the consultation.

CTA: Comment DENTAL for the full 10-video list. Carousel · MOFU

1.3 Content that builds trust before the consultation

Built on: Jakeyla Reed 'sell your patients before the consultation', 'Content ideas for a Doctors Office' (answer embarrassing questions, show BTS). Their gap: no consultation flow shown.

1.3.1 Show what happens in your consultation room in 45 seconds. It removes the #1 fear of booking.

On-screen: “Inside the consultation room”

- The unknown is scarier than the price. Show: arrival, assessment, the photos, the plan, no pressure.
- Use clinic B-roll + voiceover, so the doctor doesn't need to perform.
- End line: 'No one will push you into anything here.'

CTA: Save this format for your clinic. Clinic B-roll + VO · MOFU

1.3.2 'What I'd do if this was my own skin': the most trusted format a doctor can post.

On-screen: “If it were my own skin...”

- It's personal, honest and specific. Patients feel they're getting insider advice.
- Example: 'If I had melasma in Dubai's summer, here's my exact plan.'
- Keep it evidence-based; add the source in the caption.

CTA: Comment OWN for 5 more 'if it were me' prompts. Talking head · TOFU

1.3.3 Recovery-day reality videos: why showing downtime honestly increases bookings.

On-screen: “Day 1 → Day 7, honestly”

- Patients fear the unknown downtime more than the downtime itself.
- Show the realistic day-by-day (with consent), including the 'ugly' day.
- Honesty reduces regret, complaints and no-shows.

CTA: Follow for more honest clinic content. Day-by-day montage · MOFU

1.4 Seasonal and local content calendars (UAE vs. USA)

Built on: 'Clinic content calendar' cluster ('Plan a month of content in 45 min'). Their gap: no categories, no examples, no market-specific seasonality. Your gap: UAE + US calendars.

1.4.1 Dubai summer skin plan: the posts your patients are searching for in June.

On-screen: “Dubai summer = these 6 posts”

- Heat, humidity and sun drive pigmentation, melasma, sweat and acne questions.
- List 6 post ideas on screen, mapped to the treatments the clinic wants to sell in summer.
- Publish 4–6 weeks before demand peaks.

CTA: Comment SUMMER for the full calendar.

Carousel calendar · TOFU

1.4.2 Wedding season, Ramadan and Eid: the content calendar UAE aesthetic clinics keep missing.

On-screen: “UAE clinic calendar: 3 peaks”

- Pre-wedding skin and smile plans start 3–6 months out. Post accordingly.
- Ramadan: gentle, respectful content (routine changes, hydration, timing treatments).
- Pre-Eid glow-up content: book-ahead messaging, not discounts.

CTA: Save this. Plan it before the season starts.

Talking head · MOFU

1.4.3 US clinics: 'new year, new smile' and pre-summer content, planned 6 weeks early.

On-screen: “US aesthetic calendar”

- January: smile and skin resolutions. Spring: body and pre-summer. Fall: laser season (less sun).
- Holiday parties: quick-win treatments (fillers, facials) booked by November.
- Show a 12-month grid on screen.

CTA: Comment CALENDAR for the 12-month grid.

Calendar walkthrough · MOFU

1.5 What NOT to post

Built on: 'Stop putting your patients to sleep with the same 5 Symptoms videos', 'Bad / Good / Excellent' content reels. Their gap: no explanation of WHY, no replacement. You give the swap.

1.5.1 5 posts that make a dermatologist look like a skincare influencer and lose patient trust.

On-screen: “5 posts that cost you trust”

- Product hauls without clinical context; trend dances with no message; 'shelfie' routines.
- Discount flyers and 'Happy Friday' graphics.
- For each one, show the doctor-level replacement.

CTA: Send this to a derm friend.

List + examples · TOFU

1.5.2 Stop posting 'Happy Monday' flyers. Here's what to replace each one with.

On-screen: “Flyer ❌ → Video ✅”

- Generic flyers say 'we have a designer', not 'we have expertise'.
- Show 3 real flyer types (anonymised) next to a 1-line video replacement for each.
- Rule: if any clinic could post it, don't.

CTA: Comment SWAP for 10 more replacements.

Before/after posts · TOFU

1.5.3 Why your 'meet the team' photo gets likes but zero DMs, and the 20-second fix.

On-screen: “Likes ≠ DMs”

- Team photos create warmth but give no reason to book.
- The fix: each team member answers ONE patient question on camera (20 seconds).
- Now the team post builds trust and answers objections.

CTA: Try it this week and tag me.

Teardown · MOFU

PILLAR 2 OF 10 · 10% OF POSTS

Hooks, Scripting & Value-Per-Second (The Craft)

Shows you understand why medical videos die in 3 seconds. It's the skill clinics pay you for.

Research basis: Keyword clusters: 'doctor content strategy', 'Doctors personal branding' ('Reel hooks for doctors'). Competitors: Akash Gupta (QUEST, 'comment HOOK'), Amra Farooq (visual hooks, ChatGPT vs. authentic scripts), 'How to script reels as a doctor'. Their gap: no worked rewrites, hooks withheld, frameworks without examples.

2.1 Hook engineering

Built on: Akash Gupta 'scroll-stopping hooks' (promised 20 hooks, showed none) and Amra Farooq 'visual hooks'. Your gap: show the hooks and explain why each works.

2.1.1 Your hook isn't the first sentence. It's the first frame, the first word and the first text line, all at once.

On-screen: “Hook = Visual + Written + Verbal”

- Visual hook: movement or an unexpected object in frame 1 (a laser tip, a mould of teeth).
- Written hook: 5–7 words, specific. Verbal hook: the same promise, said in under 2 seconds.
- Show one video where all three mismatch, then the fixed version.

CTA: Save this checklist for your next filming day.

Visual breakdown · TOFU

2.1.2 I rewrote a dermatologist's hook 5 ways. Here's which one patients would actually stop for.

On-screen: “1 hook, 5 rewrites”

- Original: 'Today let's talk about acne.' Rewrites: number, fear, myth, POV, result-first.
- Explain which suits which goal (reach vs. bookings).
- Pick the winner and explain why: specificity + patient language.

CTA: Comment HOOK and I'll rewrite yours.

Split screen · TOFU

2.1.3 Doctors: stop starting videos with 'Hi, I'm Dr...'. Say this in your first 1.5 seconds instead.

On-screen: “Delete your intro”

- Your name is in the caption and the profile. The first seconds belong to the patient's problem.
- Formula: [patient's thought] + [surprising answer].
- Example: 'Your SPF isn't failing. You're using a quarter of the amount you need.'

CTA: Follow for one hook fix a day.

Talking head · TOFU

2.2 Script surgery (live rewrites)

Built on: 'How to script Instagram reels as a doctor' (framework, then a long melanoma example) and 'ChatGPT-sounding scripts'. Your gap: cut a real script on screen.

2.2.1 This 60-second veneer script had 22 seconds of filler. Watch me cut it.

On-screen: “60s → 38s, same message”

- Highlight filler in red: greetings, 'basically', repeating the question, the long outro.
- Apply the delete test sentence by sentence.
- Play both versions back. The tighter one feels more confident and expert.

CTA: Comment SCRIPT and send me yours to cut.

Screen recording · MOFU

2.2.2 Turning a boring 'benefits of retinol' video into a story patients finish.

On-screen: “Boring → binge-worthy”

- Lists of benefits don't hold attention; a patient problem with a turn does.
- Rewrite as: patient worry → mistake → doctor's fix → result timeline.
- Keep the same facts, just re-ordered.

CTA: Save this structure.

Before/after script · TOFU

2.2.3 The 4-line script structure I use for every doctor: hook, stakes, payload, next step.

On-screen: “Hook → Stakes → Payload → Next step”

- Hook: the patient's thought. Stakes: why it matters now.
- Payload: 3 steps or 1 demo, one idea per line.
- Next step: one CTA tied to the payload. Show a filled-in example.

CTA: Screenshot this template.

Template on screen · MOFU

2.3 Retention and pacing

Built on: Akash Gupta QUEST retention framework (no example), 'Memorable medical content' thought-experiment reel.
Your gap: show where viewers drop off and why.

2.3.1 Why patients scroll at second 7, and the 'open loop' that keeps them watching.

On-screen: “Second 7 = the danger zone”

- After the hook, most videos 'explain the context'. That's where viewers leave.
- Open loop: tell them the best point comes last ('and #3 is the one most clinics get wrong').
- Show a retention graph (dummy or your own) with the drop and the fix.

CTA: Follow. Part 2 is on pacing cuts.

Talking head + graphic · TOFU

2.3.2 On-screen text shouldn't repeat what you're saying. Here's what it should do instead.

On-screen: “Text ≠ subtitles”

- Captions carry your words. Big text should carry the structure: 1/3, ❌/✅, the number.
- Side by side: duplicated text vs. structural text.
- Structural text helps people watching on mute and lifts saves.

CTA: Save for your editor.

Demo · TOFU

2.3.3 The 'one idea per breath' rule for doctors who over-explain.

On-screen: “One idea. One breath.”

- Doctors are trained to be thorough; Reels reward clarity.
- Each sentence = a claim, a number, an example or a step.
- Read a long sentence, then split it into 3 breaths. Instantly clearer.

CTA: Tag a doctor who over-explains 😄

Talking head · TOFU

2.4 Outlier research (templatising winners)

Built on: your File 5 tonality reel (5x outlier rule, 70/20/10) and Amra Farooq's 'content research system' (website withheld). Your gap: show the actual research process openly.

2.4.1 How I find viral dermatology videos to model: the 5x outlier rule.

On-screen: “Followers × 5 = outlier”

- If an account has 100k followers and a video hits 500k views, that's an outlier.
- Screen-record the search: specialty creators with large audiences, sorted by views.
- Document the visual, written and verbal hook, the format and the angle.

CTA: Comment OUTLIER for my research sheet template. Screen recording · MOFU

2.4.2 Copy a viral format without copying the content: the 70/20/10 rule for doctors.

On-screen: “70 / 20 / 10”

- 70% proven outlier structures from top creators in the specialty.
- 20% doubling down on your own winners (templatise them).
- 10% experiments: reactions, transformations, new formats.

CTA: Save this. Use it every month. Talking head · MOFU

2.4.3 One outlier video → 10 scripts for a cosmetic dentist, built in front of you.

On-screen: “1 outlier → 10 scripts”

- Pick an outlier; keep the hook structure and format.
- Swap the topic 10 times: veneers, whitening, bonding, aligners, implants...
- Show 3 finished hooks on screen.

CTA: Comment 10 for the full list. Screen recording · MOFU

2.5 Camera confidence for doctors

Built on: 'Camera confidence for doctors' ('Confidence comes after execution') and the 'I don't have time' objection reel. Their gap: no exercise, no system. Your File 1 lists 'remove the fear of camera' as a topic you love.

2.5.1 Doctors who hate the camera: film in 'consultation mode', not 'presenter mode'.

On-screen: “Talk to ONE patient”

- Doctors are great at explaining to one patient; they freeze when 'presenting'.
- Trick: someone sits behind the camera and asks the question; the doctor answers them.
- Show the stiff version, then the consultation-mode version.

CTA: Try it in your next filming session. Talking head · TOFU

2.5.2 How a busy dermatologist films 40 videos in two 3-hour sessions a month.

On-screen: “40 videos. 6 hours.”

- Pre-written scripts, a teleprompter, 3 outfit changes, one background per batch.
- Order: easy talking heads first, B-roll last.
- The doctor never thinks about content between sessions; you handle the rest.

CTA: Comment BATCH for the filming-day checklist. BTS of filming day · MOFU

2.5.3 You don't need to be charismatic on camera. You need to be clear.

On-screen: “Clear > charismatic”

- Patients aren't looking for entertainment; they're looking for safety and clarity.
- Three habits: short sentences, look at the lens, one point per video.
- Your calm 'doctor voice' is the brand.

CTA: Send this to a doctor who says 'I'm not a camera person'. Talking head · TOFU

PILLAR 3 OF 10 · 15% OF POSTS

Views → Patients (Content as a Patient-Acquisition Channel)

Separates you from 'grow your Instagram' creators. Doctors want booked consultations, not followers.

Research basis: Keyword clusters: 'patient acquisition strategies', 'patient inquiry conversion', 'Clinic lead generation / clinic marketing ROI'. Competitors: Amra Farooq ('relevance over reach', 'patients may already be watching'), Joshua Navaro (Orange County case study), Eshaan (profile conversion). Their gap: no conversion data, no attribution, no audits.

3.1 Views vs. patients

Built on: Amra Farooq 'A reel can get millions of views and still generate zero patients' (no named example or conversion data). You add numbers and a pre-posting check.

3.1.1 A 200k-view Reel with zero consultations is a failed Reel. Here's how to tell before you post.

On-screen: “200k views. 0 patients.”

- Ask 3 questions before posting: Is it for MY local patient? Does it show expertise? Is there a next step?
- Viral skincare myths attract a global audience; local treatment content attracts bookers.
- Show the 3-question checklist on screen.

CTA: Save the checklist. Use it before every post.

Talking head · TOFU

3.1.2 The 3 numbers I check before I check views: profile visits, DMs, link taps.

On-screen: “Views are last”

- Profile visits = curiosity. DMs = intent. Link taps = action.
- Screen-record Instagram Insights (your own or a demo account).
- A Reel with fewer views but more DMs is the better Reel.

CTA: Follow for the full metrics series.

Insights screen recording · MOFU

3.1.3 Your viral skincare-myth video brought followers from 40 countries and no patients from Dubai.

On-screen: “Wrong audience, right content?”

- Broad topics bring broad audiences. Fine for reach, useless for bookings alone.
- Fix: localise ('in Dubai's humidity...'), name the treatment you offer, add a local CTA.
- Mix: some reach videos, most local-intent videos.

CTA: Comment LOCAL for 10 localised hooks.

Talking head · TOFU

3.2 Profile conversion

Built on: your File 5 'bio doesn't pass the 5-second rule' reel, 'Most doctors lose potential patients before they watch a Reel', Eshaan (profile visitors). Their gap: no before/after profiles shown.

3.2.1 Your bio fails the 5-second test. Line-by-line rewrite for a cosmetic dentist.

On-screen: “Bio: before → after”

- L1: who you are + specialty + city (searchable name field).
- L2: the transformation you deliver. L3: proof (years, cases, credentials).
- L4: one clear CTA to book or DM.

CTA: Comment BIO and I'll rewrite yours.

Profile teardown · MOFU

3.2.2 Highlights are your sales page. The 5 highlights every aesthetic clinic needs.

On-screen: “5 highlights = 1 sales page”

- 1) Results (with consent). 2) Treatments explained. 3) Meet the doctor.
- 4) FAQs and prices policy. 5) Reviews/testimonials.
- Show a clinic profile with messy highlights vs. organised ones.

CTA: Save this and fix your highlights tonight. Screen recording · MOFU

3.2.3 Pinned posts: the 3 videos a new patient should see first.

On-screen: “Pin these 3”

- Pin 1: who the doctor is and why trust them. Pin 2: signature treatment explained.
- Pin 3: a result/story or review compilation.
- New visitors decide in seconds. Pins are your first impression.

CTA: Follow for more profile fixes. Talking head · MOFU

3.3 Calls-to-action that patients actually use

Built on: competitor CTA analysis. Most reels in your data have 'No clear CTA' or vague ones ('DM me'). Your gap: specific, value-continuing CTAs.

3.3.1 'Link in bio' is dead for clinics. Use this DM-keyword CTA instead.

On-screen: “Link in bio ❌ → Keyword ✅”

- Keyword comments ('comment GLOW') trigger an instant DM, which starts a conversation.
- Conversations convert better than cold links.
- Show the comment → auto DM → question → booking flow.

CTA: Comment KEYWORD and watch what happens 😉 Demo · MOFU

3.3.2 Soft CTA vs. hard CTA: when a dermatologist should ask for the booking.

On-screen: “When to ask for the booking”

- Discovery content: soft CTA (save, follow).
- Treatment and proof content: hard CTA (book, DM).
- Asking for the booking on every post feels salesy; never asking wastes intent.

CTA: Save this rule. Talking head · MOFU

3.3.3 The comment-keyword → instant reply → booking flow, shown end to end.

On-screen: “Comment → Reply → Booked”

- Screen-record a demo: comment 'LASER' → DM in seconds → 2 qualifying questions → 2 time slots.
- Point out what the AI says and when a human takes over.
- This is where content meets your AI systems.

CTA: Comment FLOW if you want this built for your clinic. Screen recording · BOFU

3.4 Tracking and attribution

Built on: Joshua Navaro (ROI claims without timeframe or attribution) and Dr. Nickc KPI teaser. Your gap: show HOW you know which video brought which patient.

3.4.1 How to know which Reel brought which patient, without expensive software.

On-screen: “Which Reel = which patient?”

- Unique comment keywords per Reel. Ask 'how did you hear about us?' at intake.
- A simple sheet: date, source, Reel, treatment, booked Y/N, value.
- Review monthly and make more of what books.

CTA: Comment SHEET for the tracking template. Talking head + sheet · MOFU

3.4.2 The one question your front desk must ask every new patient.

On-screen: “'How did you hear about us?'”

- Without it, every marketing decision is a guess.
- Make it a required field in booking and the intake form.
- Be specific: 'Instagram, which video?'

CTA: Send this to your front desk. Talking head · MOFU

3.4.3 My monthly clinic growth report: what I show doctors instead of 'we posted 40 Reels'.

On-screen: “Activity ≠ results”

- 3 sections: content metrics, patient-acquisition metrics, AI/retention metrics.
- Lead with inquiries, bookings and reactivated patients.
- Show a dummy report page.

CTA: Comment REPORT for the template. Report walkthrough · BOFU

3.5 Content → inquiry economics

Built on: 'New-patient acquisition math', 'Cost per patient (£400...)' and Akash Gupta's revenue estimates (unsubstantiated). Your gap: transparent math with stated assumptions.

3.5.1 If one veneer case is worth $8,000, how many views do you actually need? Let's do the math.

On-screen: “$8,000 ÷ views = ?”

- Walk the funnel with clearly labelled assumptions: views → profile visits → DMs → consults → cases.
- Show it isn't about millions of views; it's about the right few thousand.
- Tell viewers to plug in their own numbers.

CTA: Comment MATH for the calculator. Whiteboard · MOFU

3.5.2 Small accounts book more patients than big ones. Here's why local relevance wins.

On-screen: “Small account. Full calendar.”

- A local doctor with 3k engaged followers can outbook a 300k generic account.
- Local search, local language, local treatments.
- Relevance > reach.

CTA: Follow if you're a small account with big skills. Talking head · TOFU

3.5.3 1 topic → 6 videos → 1 consultation: how a content cluster sells a high-ticket treatment.

On-screen: “Content cluster”

- Topic: Morpheus8. Videos: what it is, who it's for, pain, downtime, results timeline, myths.
- A patient who watches 3+ arrives pre-sold.
- Show the cluster as a diagram.

CTA: Save this to plan your next treatment launch. Diagram · MOFU

PILLAR 4 OF 10 · 10% OF POSTS

Patient Psychology: How Patients Choose a Dermatologist or Cosmetic Dentist

Your clinical edge. You explain patient fear, trust and decisions with a doctor's authority.

Research basis: Keyword clusters: 'patient inquiry conversion' ('Price and patient compliance', 'Patient proof and trust'), 'Dentists Instagram marketing' ('WORDS REALLY MATTER', 'premium dental pricing'). Competitors: Shivam Bansal ('brain chooses who feels safe'), Jakeyla Reed (pre-sell), Maha Chalawi (luxury clients, price shoppers). Their gap: no specialty examples, no pricing language, no consult flow.

4.1 The fears behind high-ticket treatments

Built on: Shivam Bansal '2 Gynaes: 1 booked, 1 isn't' and Sam R Patel 'They made me feel understood'. Your gap: name the specific fears per treatment.

4.1.1 'Will it look fake?' The fear stopping your filler patients, and the video that removes it.

On-screen: "'Will it look fake?'"

- Name the fear in the patient's own words.
- Show the answer: subtle approach, staged treatment, the doctor's philosophy.
- Content idea for the clinic: a 'natural results' series.

CTA: Comment NATURAL for the series outline. POV + talking head · TOFU

4.1.2 Dental anxiety is why veneer inquiries stall. 3 videos that calm it before the visit.

On-screen: "Dental anxiety = stalled bookings"

- Video 1: 'What you'll feel at each step'. Video 2: the clinic tour.
- Video 3: 'Questions anxious patients are too shy to ask'.
- Reducing fear raises inquiry → booking rates.

CTA: Send this to a cosmetic dentist. Talking head · MOFU

4.1.3 Price isn't the objection. Regret is. How to address regret in your content.

On-screen: "Price ❌ Regret ✅"

- High-ticket patients fear wasting money on the wrong doctor more than the price itself.
- Content that reduces regret: candidacy, realistic results, aftercare plan.
- Make 'what if I don't like it?' a video.

CTA: Save this mindset shift. Talking head · MOFU

4.2 Trust signals patients look for

Built on: 'Two doctors, same city, same skills' (88% stat unsubstantiated) and 'Posts every doctor should have on their profile'. Your gap: show the actual trust signals on real-style profiles.

4.2.1 Patients decide in seconds on your profile. These are the trust signals they scan for.

On-screen: "Trust scan in 7 seconds"

- Doctor's face, credentials, city, real results, reviews, a clear CTA.
- Walk through a profile, marking each signal ✅/❌.
- Missing signals = lost patients, even with great content.

CTA: Comment SCAN and I'll check your profile. Profile walkthrough · TOFU

4.2.2 Why patients trust your face more than your clinic logo.

On-screen: “Face > logo”

- People trust people. Clinic-logo-only accounts feel corporate.
- Doctor-led content builds familiarity before the visit.
- Clinic accounts should feature their 4–5 specialists (your clinic-branding model).

CTA: Follow for clinic-branding tips. Talking head · TOFU

4.2.3 Credentials vs. clarity: which builds more trust on Instagram?

On-screen: “Credentials or clarity?”

- Credentials get you considered; clarity gets you chosen.
- Use credentials as one line of proof, not the whole message.
- Example rewrite of a credential-heavy caption.

CTA: Save this. Talking head · TOFU

4.3 Premium positioning without discounts

Built on: Maha Chalawi ('Don't attract price shoppers', no examples), 'How to charge premium as a dentist', 'Premium pricing without attrition'. Your gap: specific positioning language for physicians.

4.3.1 Stop attracting price shoppers. Content that attracts patients who value expertise.

On-screen: “Price shoppers vs. value patients”

- Discount posts attract people who'll leave for the next discount.
- Expertise content (process, safety, judgment) attracts loyal patients.
- Show 3 caption swaps.

CTA: Comment PREMIUM for 10 premium-positioning hooks. Talking head · MOFU

4.3.2 Why discount flyers quietly damage a dermatologist's reputation.

On-screen: “The hidden cost of '30% OFF'”

- Medical expertise and 'sale' messaging send conflicting signals.
- Alternatives: packages, memberships, seasonal treatment plans.
- Keep value high and the brand clinical.

CTA: Send this to a clinic owner. Talking head · TOFU

4.3.3 How luxury clinics in Dubai talk about price, and what US clinics can learn.

On-screen: “Dubai luxury language”

- The language of luxury clinics: consultation-first, personalised plans, discretion.
- US clinics can borrow the experience language.
- Compare 2 price replies: commodity vs. premium.

CTA: Follow for more Dubai × USA comparisons. Comparison · MOFU

4.4 Decision journey and timing

Built on: 'Lead-to-patient system', 'Patient journey conversion: smooth journey beats larger audience'. Your gap: map the weeks of watching before a DM.

4.4.1 Most patients watch you for weeks before they DM. Here's what they need to see in that time.

On-screen: “The silent watchers”

- Week 1: discovery (myths, education). Week 2–3: trust (process, doctor, results).
- Week 4: proof + clear next step.
- Your content mix should cover every stage every week.

CTA: Save this journey map. Diagram · MOFU

4.4.2 The 'research phase' patient: content for people comparing 3 clinics.

On-screen: “They're comparing you”

- Comparison content: 'what to ask any clinic before booking X'.
- It positions you as the honest guide, and the obvious pick.
- Include the questions on screen.

CTA: Comment ASK for the question list. Talking head · MOFU

4.4.3 The patient who asks 'how much?' isn't cold. How to answer without losing them.

On-screen: “'How much?' = interest”

- Price questions are buying signals.
- Reply formula: range or 'from' price + what affects it + invite to a consult with 2 time options.
- Show a bad reply and a good reply.

CTA: Send this to your front desk. Script demo · MOFU

4.5 Doctor–patient communication (your medical edge)

Built on: 'WORDS REALLY MATTER in a dental practice', 'Message framing psychology'. Your gap: a doctor explains communication from medical training.

4.5.1 What medical school taught me about patient trust that marketers don't know.

On-screen: “What marketers don't learn”

- Patients remember how they felt, not what you said.
- Breaking bad news, consent and active listening apply to content too.
- Your story: why being a doctor makes you better at this.

CTA: Follow a doctor who gets marketing. Talking head · TOFU

4.5.2 Words that scare patients in clinic videos, and what to say instead.

On-screen: “Scary words → safe words”

- 'Needle', 'cut', 'pain' versus clearer, honest alternatives (without hiding the truth).
- Medical jargon makes patients feel stupid; plain language makes them feel safe.
- Show 5 swaps on screen.

CTA: Save these swaps. List · TOFU

4.5.3 Explaining risks honestly on Instagram makes you more bookable, not less.

On-screen: “Risks = trust”

- Patients already know nothing is risk-free; hiding risks feels like selling.
- Honest risk content builds confidence and reduces complaints.
- Template: what can happen, how often (roughly), what we do to prevent it.

CTA: Comment HONEST for the template. Talking head · MOFU

PILLAR 5 OF 10 · 5% OF POSTS

Evidence-Based & Compliant Medical Marketing (UAE + USA)

Your most defensible gap: 'doctor-reviewed, evidence-based, compliant' content that generic agencies can't offer.

Research basis: Keyword cluster: 'Dermatologist/Hair transplant/Cosmetic surgeons personal branding' ('Medical advertising compliance'). Competitors: Amra Farooq ('comment Checklist', withheld), Akash Gupta (NMC testimonials), Shivam Bansal ('scared of posting the wrong thing', NMC). Their gap: India-only rules, no examples, no jurisdiction detail. Many white spaces in your research say 'add compliance disclaimer'. ⚠ Verify every rule with the regulator's current guidance before filming.

5.1 UAE medical advertising rules

No competitor in your dataset covers DHA / DoH / MOHAP. This is an open lane for you.

5.1.1 Running a clinic in Dubai? Your Instagram ads may need approval first. What to check with DHA.

On-screen: “Dubai clinic ads: check this first”

- Health advertising in the UAE is regulated, and some content may need approval before it's published.
- List what to check: who approves, what counts as an advert, what's restricted.
- Disclaimer: educational, not legal advice; confirm with DHA.

CTA: Follow for UAE clinic-marketing rules made simple. Talking head + carousel · MOFU

5.1.2 Dubai vs. Abu Dhabi vs. other emirates: who regulates your clinic's marketing?

On-screen: “DHA · DoH · MOHAP”

- A simple map: which authority covers which emirate.
- Why clinics with branches in several emirates need to check each one.
- Encourage owners to keep approvals on file.

CTA: Save this map. Carousel · MOFU

5.1.3 Doctors posting paid collaborations in the UAE: what to know about advertiser permits.

On-screen: “Paid posts in the UAE?”

- The UAE regulates paid promotional content by creators.
- What doctors should check before accepting brand deals.
- Keep it factual and point to official sources.

CTA: Send this to a doctor who does brand deals. Talking head · MOFU

5.2 USA rules (FTC, HIPAA, state boards)

Not covered by any competitor in your dataset (they are India/UK focused). This matches your US market.

5.2.1 That patient selfie you reposted might be a HIPAA problem. What written authorization should cover.

On-screen: “Repost = risk?”

- Identifiable patient images and stories in marketing generally need written authorization.
- What the form should cover: what's used, where, for how long, and how to revoke.
- Safer alternatives: anonymised, consented, or doctor-only content.

CTA: Save this before your next repost. Talking head · MOFU

5.2.2 'Cosmetic dentistry specialist': why that phrase can get US dentists in trouble.

On-screen: “Careful with 'specialist'”

- Many state boards restrict 'specialist' claims to recognised specialties.
- Cosmetic dentistry is commonly not one of them, so check your state's rules.
- Safer wording examples.

CTA: Send this to a cosmetic dentist. Talking head · MOFU

5.2.3 Fake or incentivised reviews: what the FTC's rules mean for clinics asking for Google reviews.

On-screen: “Reviews: the rules”

- Fake reviews and review suppression are prohibited.
- Be careful with incentives; ask every patient, not just the happy ones.
- Never reveal patient details when replying.

CTA: Save this checklist. Carousel · MOFU

5.3 Before/after and testimonial ethics

Built on: Dr. Nickc 'perfect patient testimonial' (no consent detail), 'Storytelling before-and-afters', 'Patient filming ethics'. Their gap: consent and honesty missing.

5.3.1 Before/afters that build trust vs. ones that break rules: same lighting, same angle, real consent.

On-screen: “Honest before/after”

- Same lighting, angle, expression, no filters or retouching.
- Written consent, and say how many sessions/time between photos.
- Show a misleading pair and an honest pair.

CTA: Save this standard. Split screen · MOFU

5.3.2 How to film a patient testimonial with consent that holds up in the US and UAE.

On-screen: “Testimonial consent checklist”

- Written consent before filming; explain where it'll be used.
- Let the patient review it; offer a right to withdraw.
- Interview questions that get honest, specific stories.

CTA: Comment TESTIMONIAL for the checklist. Checklist walkthrough · MOFU

5.3.3 'Results may vary' isn't enough. How to present outcomes honestly.

On-screen: “Beyond 'results may vary'”

- Context: number of sessions, timeframe, candidacy.
- Show a range of results, not only the best case.
- Honesty is both compliant and persuasive.

CTA: Follow for ethical marketing that still sells. Talking head · TOFU

5.4 Evidence-based content

Built on: dermatology product reels (seeds for skin, ingredient guides) flagged in your research as 'needs evidence level / safety context'. Amra Farooq 'ChatGPT scripts'. Your gap: visible fact-checking.

5.4.1 I fact-check every script we make. Here's what that process looks like.

On-screen: “Doctor-reviewed. Every script.”

- Claim → source (guidelines, reviews, trials) → wording check → compliance check.
- Screen-record the process with one example claim.
- This is why our content is different from generic agencies'.

CTA: DM REVIEW if you want doctor-reviewed content.

BTS screen recording · BOFU

5.4.2 Viral skincare claims dermatologists should stop repeating, and what the evidence says.

On-screen: “Viral ≠ evidence”

- Pick 3 popular claims and give the honest, evidence-based answer.
- Show how to say 'the evidence is limited' without sounding boring.
- Add sources in the caption.

CTA: Save and share with a derm.

Myth-bust · TOFU

5.4.3 Why ChatGPT-written medical scripts are risky, and how doctors should use AI safely.

On-screen: “AI scripts: the risk”

- AI can invent facts, sound generic and miss local rules.
- Safe use: AI for structure and ideas; the doctor for facts and voice.
- Always fact-check before posting.

CTA: Follow for safe AI for doctors.

Talking head · TOFU

5.5 Claims and wording

Built on: compliance white spaces across your research ('substantiate claims', 'avoid implying universal results').

5.5.1 'Painless', 'permanent', 'guaranteed': words to remove from your clinic content today.

On-screen: “3 words to delete”

- Absolute claims are risky and usually untrue.
- Swap for honest alternatives: 'minimal discomfort', 'long-lasting', 'expected results'.
- Show 5 caption fixes.

CTA: Save this list.

List · TOFU

5.5.2 Rewriting 5 risky aesthetic claims into compliant ones that still sell.

On-screen: “Risky → compliant → still sells”

- Split screen: original claim and rewrite.
- Explain why each rewrite is both safer and more believable.
- Keep it specific to injectables, lasers and dental.

CTA: Comment CLAIMS for 20 more rewrites.

Split screen · MOFU

5.5.3 Educational vs. promotional content: why the line matters for doctors.

On-screen: “Education or advertising?”

- Promotional content often carries more rules (approval, disclaimers).
- Educational content builds trust without hard selling.
- How to mix both safely.

CTA: Follow for more.

Talking head · MOFU

PILLAR 6 OF 10 · 10% OF POSTS

The Patient Leak: Inquiry → Booking (Speed, DMs, Front Desk)

Your bridge between content and AI: 'Content gets you the opportunity; your systems decide what happens to it.'

Research basis: Keyword clusters: 'patient inquiry conversion', 'Ai receptionist for medical clinics' (front desk), 'patient acquisition strategies' ('most expensive sound is the phone ringing', 'offer two slots', 'where are you losing patients'). Competitors: Sam R Patel (front desk, no fixes or metrics), Dr. Nickc ('comment FOLLOWUP', withheld). Their gap: no live tests, no scripts, no AI layer.

6.1 Speed to lead

Built on: 'MedSpa owners, if your front desk takes forever to reply...', 'Most clinics lose patients because no one answers fast enough'. Your gap: real (anonymised) tests with timestamps.

6.1.1 I messaged 10 Dubai aesthetic clinics at 9 pm. Here's how many replied before morning.

On-screen: “9:00 PM → ? replies”

- Send a genuine inquiry; book nothing; anonymise all names.
- Show the timestamps and response quality.
- Lesson: the clinics that reply fast, ask a question and offer slots win.

CTA: Comment LEAK and I'll test your clinic.

Experiment series · TOFU

6.1.2 Every hour your clinic waits to reply, a patient books somewhere else.

On-screen: “Slow reply = lost patient”

- High-intent patients message several clinics at once.
- The first helpful reply usually wins.
- Fix: instant acknowledgement + a qualifying question + a human follow-up.

CTA: Send this to your front desk manager.

Talking head · MOFU

6.1.3 After-hours inquiries are your highest-intent patients. Who answers them right now?

On-screen: “Who answers at 11 pm?”

- Patients research at night, after work and kids.
- No reply until morning means the intent cools.
- Options: on-call staff vs. AI first response with human handover.

CTA: Comment NIGHT for the after-hours flow.

Talking head · MOFU

6.2 DM and WhatsApp conversion

Built on: 'Handling patient price questions', 'Price-estimate inquiries', 'Leads slipping through the cracks (WhatsApp)'. Their gap: no exact scripts.

6.2.1 The 'How much is Botox?' DM: 3 replies that lose the patient, 1 that books them.

On-screen: “'How much?' — 4 replies”

- ❌ Just a price. ❌ 'Come in for a consultation.' ❌ No reply.
- ✅ Range + what affects price + a question + 2 slot options.
- Show the screenshots.

CTA: Save and train your team with it.

Screen rec + script · MOFU

6.2.2 WhatsApp is the UAE's front desk. How clinics should structure replies.

On-screen: “WhatsApp = front desk”

- Greeting with the clinic name, quick acknowledgement, qualifying question.
- Use templates, quick replies and labels for follow-up.
- Opt-in and privacy basics.

CTA: Follow for UAE clinic tips. Demo · MOFU

6.2.3 Don't send a price list. Send this 3-message sequence instead.

On-screen: “Price list ❌ 3 messages ✅”

- Message 1: warm reply + one question about their concern.
- Message 2: tailored info + a short video link from the doctor.
- Message 3: 2 appointment options + reassurance.

CTA: Comment SEQUENCE for the copy. Script on screen · MOFU

6.3 Phone and front-desk scripts

Built on: Sam R Patel (best/worst greetings, exceptional call, 4 costly mistakes) and 'Stop asking what time works'. Their gap: no fixes or downloadable script; blames the receptionist.

6.3.1 Stop asking 'When would you like to come in?' Offer two slots instead.

On-screen: “Two slots > open question”

- Open questions create decision fatigue and delay.
- 'I have Tuesday 4 pm or Thursday 11 am. Which suits you better?'
- Role-play both versions.

CTA: Send this to your receptionist. Role-play · MOFU

6.3.2 Rating a real (anonymised) clinic call: 4 moments where the booking was lost.

On-screen: “Call review: 4 lost moments”

- Greeting, discovery, handling price, closing. Score each one.
- Show what should have been said.
- Use only calls you have permission to share, or recreate them.

CTA: Comment CALL for the scorecard. Call review · MOFU

6.3.3 Your receptionist isn't the problem. The system around them is.

On-screen: “Not the person. The system.”

- Overloaded front desks miss calls and forget follow-ups. That's normal.
- Systems: scripts, CRM tasks, reminders, AI first response.
- Owners: support your team; don't blame them.

CTA: Tag a clinic owner. Talking head · MOFU

6.4 Consult → treatment conversion

Built on: 'If I had to stop patients ghosting after consultation...', Dr. Nickc 'post-consult patient loss'. Their gap: sequence, timing and scripts not shown.

6.4.1 Patients who don't book after a consultation aren't 'no'. They're 'not yet'.

On-screen: "'Not yet' ≠ 'no'"

- Common reasons: cost planning, fear, partner's opinion, timing.
- Each needs a different follow-up message.
- Without follow-up, 'not yet' quietly becomes 'never'.

CTA: Follow for the full follow-up series. Talking head · MOFU

6.4.2 The 3-day, 7-day, 21-day follow-up that recovers 'I'll think about it' patients.

On-screen: "Day 3 · Day 7 · Day 21"

- Day 3: answer their main concern (personal message). Day 7: helpful content from the doctor.
- Day 21: gentle check-in + availability.
- Show the timeline graphic and the messages.

CTA: Comment FOLLOWUP for the messages. Timeline graphic · BOFU

6.4.3 Treatment-plan recovery: the patients already sitting in your 'pending' list.

On-screen: "Your hidden revenue list"

- Every clinic has patients with unfinished treatment plans.
- Pull the list, segment it and send a personal re-invite.
- Often the fastest revenue win in a pilot.

CTA: DM PLAN if you want this run for your clinic. Talking head · BOFU

6.5 Leak audits

Built on: 'Where are you losing patients?' (muted, unverifiable) and the 'Patient leakage points' topic. Your gap: a visible, repeatable audit.

6.5.1 Where your clinic loses patients: the 7-point leak map.

On-screen: "7 leak points"

- Content → inquiry → response → booking → show-up → follow-up → return.
- Name one typical leak at each point.
- Ask viewers to score themselves 1–7.

CTA: Comment MAP for the self-audit. Diagram · MOFU

6.5.2 I audited a cosmetic dentist's patient journey. Here are the 3 biggest leaks.

On-screen: "Audit: 3 leaks"

- Use a consenting client or an anonymised example.
- Show evidence for each leak (screenshots, timings).
- Show the fix for each.

CTA: Comment AUDIT for yours. Teardown · BOFU

6.5.3 Self-audit: 5 questions to find out if your clinic has a follow-up problem.

On-screen: "Do you have a follow-up problem?"

- 1) Who follows up non-bookers? 2) When? 3) How many times?
- 4) Is it tracked? 5) What happens after hours?
- 2+ 'I don't know' answers = a leak.

CTA: Save and answer honestly. Carousel · MOFU

PILLAR 7 OF 10 · 15% OF POSTS

AI Systems for Clinics (Receptionist, Booking, No-Shows)

Your second authority pillar. AI always tied to a clinic outcome, with clinical guardrails.

Research basis: Keyword clusters: 'Ai receptionist for medical clinics', 'healthcare chatbot / AI lead qualification', 'AI appointment booking', 'patient booking automation / no-show rate'. Competitors: Shivam Bansal (AI prescription demo, no safety), Dr. Nickc ('build systems around AI', no demo), generic 'AI agent replaces receptionist' reels. Their gap: no medical privacy, escalation, handoff or proof.

7.1 AI receptionist and missed calls

Built on: 'A client calls at 8 pm; no one picks up' (med-spa AI receptionist), 'Missed calls are missed revenue' (not medical-specific). Your gap: a medical demo with guardrails.

7.1.1 Watch an AI receptionist answer a Dubai clinic's call at 11 pm, in Arabic and English.

On-screen: “11 PM call → answered”

- A live demo (demo clinic): greeting, AI disclosure, question, booking.
- Show the language switch.
- Show the handover to a human for medical questions.

CTA: Comment AI to see it on your clinic's number. Live demo · BOFU

7.1.2 Missed-call text-back: the simplest AI system every clinic should have first.

On-screen: “Missed call → instant text”

- A missed call triggers an instant text: 'Sorry we missed you, how can we help?'
- Cheap, fast, and recovers patients who'd otherwise call the next clinic.
- Show the flow.

CTA: Save this. Start here. Demo · MOFU

7.1.3 What an AI receptionist should never say to a patient (my doctor's guardrails).

On-screen: “AI red lines”

- Never diagnose, never give medical advice, never promise results.
- Always disclose it's AI, and escalate urgent or upset patients to a human.
- Why a doctor should design these rules.

CTA: Follow for safe AI for clinics. Talking head · MOFU

7.2 Lead qualification and booking

Built on: 'Stop making your closer call 100 people to book 2', 'Fix your revenue by asking these questions'. Their gap: no rubric, no medical use case.

7.2.1 An AI that asks the 4 questions your receptionist forgets, then books the slot.

On-screen: “4 questions → booked”

- Treatment interest, timeline, previous treatments, preferred times.
- The answers are saved to the CRM so the doctor is prepared.
- Demo the chat.

CTA: Comment QUALIFY for the question set. Demo · BOFU

7.2.2 How AI qualifies a veneer inquiry without sounding robotic.

On-screen: “Human-sounding AI”

- Tone: clinic's voice, short messages, empathy lines.
- Show a robotic version and a natural version side by side.
- Always offer 'talk to the team'.

CTA: Follow for the next demo. Chat demo · MOFU

7.2.3 Instant DM reply → qualification → calendar booking: the full flow in 40 seconds.

On-screen: “DM → booked in 40s”

- Screen-record the whole flow on a demo account.
- Show the confirmation and reminder that follow.
- This is what a 'system' looks like.

CTA: DM FLOW to get this built. Screen recording · BOFU

7.3 No-shows and reminders

Built on: 'Stop sending basic appointment reminders', 'That confirmation message is vital', '5 steps to stop patients cancelling'. Their gap: timing, copy and medical use not specified.

7.3.1 Your reminder text is why patients don't show up. The 48h / 24h / day-of sequence that works.

On-screen: “48h · 24h · Day-of”

- 48h: confirm + prep instructions. 24h: reminder + easy reschedule link.
- Day-of: a friendly nudge + directions/parking.
- Show the copy on screen.

CTA: Comment REMIND for the copy. Script on screen · MOFU

7.3.2 Turning cancellations into filled slots with an automated waitlist.

On-screen: “Cancellation → filled”

- A cancellation triggers a message to waitlisted patients; first to confirm gets the slot.
- Show the flow.
- It protects the doctor's time and revenue.

CTA: Follow for more clinic automations. Demo · BOFU

7.3.3 Deposits, confirmations and reminders: reducing no-shows without annoying patients.

On-screen: “Fewer no-shows, happy patients”

- Deposits for high-value treatments set expectations.
- Make confirming easy (reply 'YES').
- Make rescheduling easy, so a cancellation doesn't turn into a no-show.

CTA: Save this. Talking head · MOFU

7.4 Safe AI in healthcare

Built on: research white spaces repeatedly saying 'add privacy, consent, escalation, compliance'. Your doctor background makes this credible.

7.4.1 AI should make your receptionist better, not replace them.

On-screen: “AI + receptionist”

- AI takes the repetitive work: FAQs, reminders, after-hours replies.
- The receptionist handles people: complex questions, upset patients, in-clinic care.
- Better patient experience, less burnout.

CTA: Send this to a worried receptionist 😊 Talking head · TOFU

7.4.2 Patient data and AI tools: questions every US clinic should ask a vendor about HIPAA.

On-screen: “Ask your AI vendor”

- Will you sign a Business Associate Agreement?
- Where is data stored, who can access it, and is it used for training?
- How is it deleted, and what happens in a breach?

CTA: Save before you buy any AI tool. Carousel · MOFU

7.4.3 Automated texts and AI calls in the US need consent. What clinic owners should know.

On-screen: “Consent first”

- US rules require consent for many automated texts and artificial-voice calls.
- Marketing messages generally need stronger consent than appointment reminders.
- Educational, not legal advice; check with counsel.

CTA: Follow for compliant clinic AI. Talking head · MOFU

7.5 What to automate (and what not to)

Built on: 'Using AI: red vs. green flag', 'Most AI in healthcare fails, the successful ones fix operations'. Their gap: no actual examples or order of implementation.

7.5.1 Don't automate everything. The 6 clinic tasks AI should handle, and the 3 it shouldn't.

On-screen: “Automate 6. Keep 3 human.”

- Automate: first response, FAQs, booking, reminders, follow-ups, review requests.
- Keep human: clinical advice, complaints, sensitive conversations.
- Show the two lists.

CTA: Save this list. List · TOFU

7.5.2 An AI receptionist won't fix a broken patient journey. Fix this first.

On-screen: “Fix the journey first”

- Automating a bad process just makes the mistakes faster.
- Map the journey, fix the scripts, then automate.
- That's how we install systems.

CTA: Comment JOURNEY for the map. Talking head · MOFU

7.5.3 The order I'd install AI in a new clinic: week 1, week 2, week 4.

On-screen: “AI install roadmap”

- Week 1: missed-call text-back + instant DM reply.
- Week 2: booking + reminders. Week 4: follow-ups, reactivation, reviews.
- Quick wins first build trust in the system.

CTA: DM ROADMAP to get yours. Roadmap graphic · BOFU

PILLAR 8 OF 10 · 5% OF POSTS

Retention, Reactivation & Reviews (Money Already in the Database)

The fastest ROI for established clinics, and the easiest visible win in a pilot.

Research basis: Keyword clusters: 'patient communication automation & reactivation campaign', 'patient retention strategies & reduce churn', 'patient database marketing', 'clinic google review / CRM'. Competitors: Sam R Patel (retention tactics ranking), Maha Chalawi (loyalty), Dr. Nickc (testimonials, GBP). Their gap: no segmentation, consent, cadence, economics or results.

8.1 Database reactivation

Built on: 'Reactivate patients who haven't booked in 2 years', 'Best & worst ways to reactivate', 'Reactivation call script'. Their gap: consent, segmentation, cadence, results.

8.1.1 You have patients who haven't visited in 12 months. Here's the first message I'd send.

On-screen: “12 months away → message 1”

- Personal, from the doctor or clinic, no discount-first.
- Reference their last treatment and give a reason to return now.
- Easy reply option + 2 time slots.

CTA: Comment REACTIVATE for the message. Script on screen · BOFU

8.1.2 Reactivation by treatment cycle: why one message for everyone fails.

On-screen: “1 message ≠ every patient”

- Injectables, facials, lasers and hygiene all have different return cycles.
- Segment by last treatment and time since the visit.
- Relevant messages = higher replies, fewer unsubscribes.

CTA: Save this. Talking head · MOFU

8.1.3 Before you run a 'we miss you' campaign, segment your list like this.

On-screen: “Segment first”

- Segments: lapsed regulars, one-time visitors, unfinished plans, consult-no-shows.
- Check consent for marketing messages.
- Show a CRM demo.

CTA: Follow for the next part. CRM demo · MOFU

8.2 Rebooking and recall

Built on: 'A strong follow-up turns one visit into long-term patients', 'Texting follow-ups improves returns'. Your gap: clinically correct recall timing explained by a doctor.

8.2.1 Recall timing that respects the treatment, explained by a doctor.

On-screen: “Right message. Right time.”

- Each treatment has a typical maintenance interval. Follow the clinic's protocol.
- Recall too early feels salesy; too late and you lose them.
- Show a sample recall table.

CTA: Comment RECALL for the table. Talking head · MOFU

8.2.2 Post-treatment follow-up on day 1, day 7 and day 30, and why each one matters clinically.

On-screen: “Day 1 · 7 · 30”

- Day 1: aftercare + 'how are you feeling?' (safety).
- Day 7: check progress, answer concerns. Day 30: result check + review ask + rebooking.
- Good care and good retention at the same time.

CTA: Save this sequence. Timeline · MOFU

8.2.3 The rebooking question your clinic should ask before the patient leaves.

On-screen: “Rebook before they leave”

- 'Your next session is ideally around [date]. Shall I hold a slot for you?'
- Patients who rebook on the spot are far more likely to return.
- Role-play it.

CTA: Send this to your front desk. Role-play · MOFU

8.3 Reviews and reputation

Built on: 'Google review request script', 'The first 50 reviews', 'QR-code patient reviews', 'Patient praise to reviews'. Their gap: compliance and automation.

8.3.1 How to ask for Google reviews without breaking the rules.

On-screen: “Ask for reviews, the right way”

- Ask everyone, at the right moment, with a direct link.
- No fake reviews, no gating, careful with incentives.
- Give a script for the team.

CTA: Save this script. Script · MOFU

8.3.2 Your first 50 reviews: the automated request flow, timed after the result shows.

On-screen: “50 reviews flow”

- Trigger: the visit is completed + time for the result to show.
- Message: thank you + a 1-tap review link.
- Reminder once, then stop.

CTA: Comment REVIEWS for the flow. Demo · BOFU

8.3.3 How to reply to a 1-star review as a doctor without breaching confidentiality.

On-screen: “Replying to 1★”

- Never confirm they're a patient or mention treatment details.
- Calm, brief, take it offline.
- Show a bad reply and a good reply.

CTA: Save this for the bad days. Talking head · MOFU

8.4 Loyalty, memberships and referrals

Built on: Maha Chalawi loyalty reels (no tier structure or economics), 'Referrals: most affordable strategy' (no script/timing). Your gap: physician-level structures + timing.

8.4.1 Loyalty programs that build value, not discount addiction.

On-screen: “Value > discounts”

- Perks: priority booking, doctor check-ins, exclusive treatments.
- Avoid training patients to wait for sales.
- Example tier structure on screen.

CTA: Comment LOYALTY for the structure. Talking head · MOFU

8.4.2 Skin memberships: how dermatology clinics turn one-off visits into recurring revenue.

On-screen: “One-off → monthly”

- A monthly membership with included treatments + member pricing.
- Predictable revenue for the clinic, a plan for the patient.
- Automate renewals and reminders.

CTA: Follow for clinic growth models. Breakdown · MOFU

8.4.3 A referral ask that feels natural, timed at the moment patients love their result.

On-screen: “Ask at the 'wow' moment”

- Timing: when the patient compliments the result.
- Script: 'If you know someone who'd benefit, we'd be glad to look after them.'
- Track referrals in the CRM.

CTA: Save this script. Script · MOFU

8.5 Lifetime value

Built on: 'Retention vs. acquisition', 'You lose clients in week one'. Their gap: no clinic math.

8.5.1 Why a $300 facial patient can be worth thousands over 3 years, and how clinics lose them.

On-screen: “$300 → $3,000+?”

- Lifetime value = visit value × visits per year × years retained (state assumptions).
- Most clinics count the first visit only.
- Where they're lost: no follow-up, no rebooking, no recall.

CTA: Comment LTV for the calculator. Whiteboard · MOFU

8.5.2 Retention beats acquisition. The math for aesthetic clinics.

On-screen: “Keep > find”

- Getting a new patient usually costs much more than keeping one.
- A small improvement in retention compounds.
- Show a simple comparison with labelled assumptions.

CTA: Save this. Whiteboard · MOFU

8.5.3 The week-one experience decides if a patient returns. What to automate in those 7 days.

On-screen: “Week one = loyalty”

- Day 0: thank you + aftercare. Day 1–2: check-in. Day 7: progress + next steps.
- Patients feel cared for, not sold to.
- Automate it so it never depends on memory.

CTA: Follow for the full retention series. Talking head · MOFU

PILLAR 9 OF 10 · 5% OF POSTS

Clinic Growth Economics, Teardowns & Proof

Makes you a strategic partner, not a vendor. Teardowns and case studies are your highest-converting BOFU content.

Research basis: Keyword clusters: 'Clinic lead generation / ROI', 'clinic growth strategy / healthcare marketing', 'clinic growth case study'. Competitors: Mrinal Tiwari (unverifiable case studies behind 'comment PDF'), Joshua Navaro (ROI without attribution), Dr. Nickc ('If you know these KPIs...', not named), Dr Sakshi (systems, no templates). Their gap: hidden numbers and unverifiable claims.

9.1 Clinic KPIs

Built on: '7 metrics every clinic owner should track', '3 KPIs for a 7-figure clinic', Dr. Nickc KPI teaser. Their gap: KPIs not named or defined.

9.1.1 The 6 numbers every aesthetic clinic owner should know every Monday.

On-screen: “6 Monday numbers”

- New inquiries, inquiry → booking %, show rate, new patients, rebooking rate, reviews.
- Define each one in a line.
- Show a simple dashboard.

CTA: Comment KPI for the dashboard.

Carousel + talking head · MOFU

9.1.2 Inquiry-to-booking rate: the KPI that tells you if marketing or follow-up is broken.

On-screen: “Marketing problem or follow-up problem?”

- Lots of inquiries + low booking rate = follow-up problem.
- Few inquiries + high booking rate = visibility problem.
- Fix the right thing first.

CTA: Save this diagnostic.

Whiteboard · MOFU

9.1.3 Cost per new patient: how to calculate it honestly (organic + paid).

On-screen: “True cost per patient”

- Include agency fees, ad spend, team time and software.
- Divide by new patients actually seen.
- Compare channels fairly.

CTA: Follow for clinic numbers made simple.

Whiteboard · MOFU

9.2 Profile and clinic teardowns

Built on: your File 5 tonality reel ('This is how I grow this dermatologist...'): a teardown format that works. Use permission or anonymise.

9.2.1 Teardown: a Miami dermatologist with 50k followers and almost no booking path.

On-screen: “50k followers, no path”

- Bio, highlights, pinned posts, CTAs: score each.
- Show 3 specific fixes.
- Anonymise or get permission.

CTA: Comment TEARDOWN to nominate a profile.

Green-screen teardown · MOFU

9.2.2 Teardown: why this Dubai dental clinic's content looks expensive but converts poorly.

On-screen: “Pretty ≠ converting”

- High production, no doctor, no patient questions answered.
- What to keep, what to change.
- A 3-video plan to fix it.

CTA: Follow for weekly teardowns.

Green-screen teardown · MOFU

9.2.3 Rating 3 cosmetic dentist profiles out of 10, and what I'd change first.

On-screen: “3 profiles. Rated.”

- The same 5 criteria for each, so it's fair and teachable.
- The highest scorer and why.
- The single biggest fix for each.

CTA: Comment RATE for yours.

Ranking · TOFU

9.3 ROI math

Built on: 'Dental practice owners want the exact math', Akash Gupta revenue estimates (no assumptions). Your gap: honest math with assumptions on screen.

9.3.1 Is a $2,000/month content + AI system worth it? Let's calculate for an implant clinic.

On-screen: “$2,000/month: worth it?”

- Average implant case value (assumption) → patients needed to break even.
- Add reactivated patients and recovered missed calls.
- Viewers plug in their own numbers.

CTA: Comment ROI for the calculator.

Whiteboard · BOFU

9.3.2 Why more leads isn't always the answer. The math of fixing conversion first.

On-screen: “More leads? Or fewer leaks?”

- Doubling leads at a 20% booking rate vs. raising booking to 40% on the same leads.
- Conversion fixes are often cheaper and faster.
- Then scale leads.

CTA: Save this.

Whiteboard · MOFU

9.3.3 Referrals vs. content vs. ads: what each patient source really costs.

On-screen: “3 sources compared”

- Referrals: high trust, unpredictable volume.
- Content: compounding, slower start. Ads: fast, stops when the spend stops.
- The best clinics use a mix.

CTA: Follow for the full breakdown.

Comparison · MOFU

9.4 Case studies (from your pilot)

Your biggest future asset. Competitors' case studies lack baseline, timeframe and attribution. Yours will include all three.

9.4.1 60 days with a [specialty] clinic: baseline, what we changed, what happened.

On-screen: “Day 0 → Day 60”

- Baseline numbers, the changes we made, the results with timeframe.
- How we attributed results (keywords, intake question).
- Client permission required.

CTA: DM PILOT if you want to be next. Case study · BOFU

9.4.2 The video that brought one clinic its most valuable consultation, and why it worked.

On-screen: “1 video. 1 big patient.”

- Show the video, its hook and structure.
- How the patient found it and what they said at intake.
- How to make more like it.

CTA: Comment WINNER for the template. Breakdown · BOFU

9.4.3 What didn't work in our first pilot, and what we changed.

On-screen: “What didn't work”

- Honest lessons build more trust than perfect results.
- One content lesson, one AI lesson, one process lesson.
- Shows you're learning fast.

CTA: Follow the build. Honest recap · BOFU

9.5 Owner strategy

Built on: 'You don't need a bigger team, you need better systems', 'Clinic owner systems', Shivam Bansal (referral collapse). Their gap: no practical prioritisation.

9.5.1 Scaling a private practice: hire more staff or build systems first?

On-screen: “Hire or systemise?”

- If the problem is repetitive work, systemise first.
- If it's clinical capacity, hire.
- Most clinics hire to fix a systems problem.

CTA: Save this decision rule. Talking head · MOFU

9.5.2 If your clinic depends 100% on referrals, your growth is left to chance.

On-screen: “100% referrals = risk”

- Referrals are great but unpredictable.
- Add a second, controllable channel: content + follow-up.
- Keep the referrals; add the predictability.

CTA: Send this to a clinic owner. Talking head · TOFU

9.5.3 Launching a new treatment? The 30-day content + follow-up launch plan.

On-screen: “30-day treatment launch”

- Week 1: awareness (what it is). Week 2: who it's for + doctor's view.
- Week 3: process + early results (with consent). Week 4: booking push + waitlist.
- AI handles the inquiries throughout.

CTA: Comment LAUNCH for the plan. Roadmap · BOFU

PILLAR 10 OF 10 · 10% OF POSTS

Dr. Azka: A Doctor Building for Doctors (Human Layer)

Turns 'an agency' into 'the doctor I trust'. Part of the 20% human side of your 80/20.

Research basis: Keyword cluster: 'healthcare business founder story, doctor turned entrepreneur'. Competitors: Amra Farooq ('This is my story :)', no turning point), Shivam Bansal (inspirational, vague), 'Doctors are born entrepreneurs' (no steps). Their gap: no tension, milestones or proof. Your File 4 origin story has real tension: doctor at 24 → business experiments → failures → AI → the gap.

10.1 Origin story

From your File 4: Healthcare → Doctor → Realisation → Experiments → Failures → AI → The gap → Helping doctors.

10.1.1 I'm a doctor at 24 and I chose not to practise clinically, yet. Here's why.

On-screen: “Doctor. 24. Different path.”

- The realisation: being a good doctor isn't the whole equation for a clinic.
- What you saw doctors struggle with: visibility, inquiries, follow-up.
- The decision to help doctors grow instead.

CTA: Follow the journey.

Talking head · TOFU

10.1.2 In medical school I realised doctors need to be online to stand out. Nobody taught us how.

On-screen: “What med school didn't teach”

- A moment from medical school that made it click.
- Patients search online before they choose.
- Why you're building what you wish existed.

CTA: Tag a doctor friend.

Story + B-roll · TOFU

10.1.3 I didn't leave healthcare. I found another way to contribute to it.

On-screen: “Not leaving. Contributing.”

- Better clinics = better patient access and experience.
- Your work helps good doctors get found.
- Mission line on screen.

CTA: Follow if you believe this too.

Talking head · TOFU

10.2 Failures and lessons

From your File 4: e-commerce attempt, previous agency model, coaching. 'My failures are actually content.'

10.2.1 My first agency failed. Here's what I got wrong.

On-screen: “My first agency failed”

- Too broad, wrong offer, no proof. Be specific and honest.
- What you learned about niching down.
- Why clinics are now your only focus.

CTA: Follow for honest business lessons.

Story · TOFU

10.2.2 I tried e-commerce before this. What it taught me about patients and customers.

On-screen: “E-commerce → clinics”

- Speed of reply, follow-up and retargeting matter in every business.
- Clinics rarely apply these lessons.
- The bridge to your AI systems.

CTA: Save this lesson.

Story · TOFU

10.2.3 Why I stopped building a business for everyone and chose only clinics.

On-screen: “Everyone → only clinics”

- You understand doctors from the inside.
- Specialisation = better results + higher trust.
- Your niche: derm, cosmetic dental, aesthetics.

CTA: Follow if you're a clinic owner.

Talking head · MOFU

10.3 Building in public

Your 2026–27 targets ($10–12k → $50k → $100k/month) make a natural public series. Share numbers you're comfortable with.

10.3.1 Day 1 of building my agency from $0 to $10k/month, working only with clinics.

On-screen: “Day 1: $0 → $10k”

- The goal, the offer, the plan for the first 60 days.
- Invite viewers to follow the weekly updates.
- Keep it real, not hype.

CTA: Follow for weekly updates.

Series · TOFU

10.3.2 Week 4 update: clients, calls, mistakes and numbers.

On-screen: “Week 4: the numbers”

- Outreach sent, replies, calls, clients signed.
- One mistake and one win.
- What changes next week.

CTA: Follow for week 5.

Series · MOFU

10.3.3 What happened when I cold-messaged 100 dermatologists.

On-screen: “100 dermatologists. Results.”

- Reply rate, best message, worst message.
- What dermatologists actually care about.
- Lesson for other founders.

CTA: Comment 100 for the message that worked best.

Story + data · MOFU

10.4 Day in the life

From your File 4: 'Day in my life as a doctor building a business for doctors', with every clip tied back to your identity.

10.4.1 A day in my life as a doctor running a content + AI agency in Dubai.

On-screen: “Doctor. Founder. Dubai.”

- Morning routine, client calls, scripting, AI builds.
- One lesson from the day at the end.
- Purple-and-white aesthetic consistency.

CTA: Follow for more days like this. Vlog · TOFU

10.4.2 Behind the scenes of scripting 40 videos for one doctor.

On-screen: “40 scripts. Behind the scenes.”

- Research, outliers, fact-checking, hooks.
- Shows the work behind the price.
- Speed it up with a timelapse.

CTA: DM SCRIPTS if you want this for your clinic. BTS · MOFU

10.4.3 A filming day at a client clinic, from our side.

On-screen: “Filming day 🎬”

- Setup, the doctor's nerves, the shot list, the wins.
- Show how easy it is for the doctor.
- Client permission required.

CTA: Follow for the results. BTS vlog · MOFU

10.5 Opinions and beliefs

From your File 4 contrarian POVs: 'Doctors don't need more followers', 'Lead problem vs. follow-up problem', 'Content gets you the opportunity...'.

10.5.1 Unpopular opinion: doctors don't need more followers. They need more patients.

On-screen: “Followers ≠ patients”

- Vanity metrics vs. patient metrics.
- A small, local, trusting audience beats a big, random one.
- What to measure instead.

CTA: Agree or disagree? Comment. Talking head · TOFU

10.5.2 Most clinics don't have a lead problem. They have a follow-up problem.

On-screen: “Lead problem? Follow-up problem.”

- Examples: slow replies, no follow-up after the consult, no reactivation.
- Fix the leaks before buying more leads.
- Ties to your AI offer.

CTA: Comment LEAK for a free audit. Talking head · MOFU

10.5.3 Content gets you the opportunity. Your systems decide what happens to it.

On-screen: “Content + Systems”

- Your signature philosophy. Repeat it often.
- Content without systems = wasted attention. Systems without content = an empty pipeline.
- That's why you do both.

CTA: Follow if this makes sense to you. Signature talk · MOFU